

State of Vermont

Secretary of State

Office of Professional Regulation

Board of Allied Mental Health

In re: KAREN GIROUX

Docket Nos. 2026-56 & 57

License No. 068.0135315

Board members present and participating:

Tammy Austin: Licensed Marriage and Family Counselor

Marjorie Trombly: Licensed Clinical Mental Health Counselor

Robert Wubbenhorst: Licensed Clinical Mental Health
Counselor

Lyn Turcotte: *ad hoc* public member

Appearances: Zoe Newman, Esq. for the State

Karen Giroux appeared without counsel and
represented herself

Presiding Officer: George K. Belcher

Summary Suspension Order

This matter came before the Board of Allied Mental Health Practitioners on a Request for a Summary Suspension. The hearing was held electronically on April 16, 2026, with the Attorney for the State; Ms. Giroux; the witnesses; and the Board members, all participating by telephone, in person, or via electronic means pursuant to the Office of Professional Regulation Rules.

Findings of Fact

Based on the evidence presented at the hearing and the arguments of counsel, the Hearing Authority finds as follows:

1. Karen Giroux, the Respondent is licensed by the State of Vermont as a Licensed Alcohol and Drug Abuse Counselor (LADAD) under license number 151.0134124. She is also licensed as a Licensed Clinical Mental Health Counselor (LCMHC) under license number 068.0135315. The Respondent was licensed in both professions during the time periods at issue here.
2. Petitions for Summary Suspension of both licenses were filed on or about April 8, 2026. Notice of hearing on the Petitions was given to the Respondent on April 8, 2026, by ordinary mail. There were no objections to notice.
3. On April 9, 2026, a Motion for Consolidation of the two cases was approved such that the Petitions for Summary Suspension were heard simultaneously by the Board of Allied Mental Health Professionals and the Administrative Law Officer acting for the Director in the profession of Alcohol and Drug Abuse Counselors.
4. The Respondent works as both a Licensed Mental Health Counselor and an Alcohol and Substance Abuse Counselor in St. Albans, Vermont. She has an estimated client load of 20-25 clients and does approximately four to five sessions per day.

Health and Fitness

5. In 2024 the Respondent suffered a dog-bite. In February 2024 she began to feel that she was not herself. In September of 2024 she began to notice that she was having “psychosis.” She also had periods of memory loss, seizures, delirium a feeling of Alzheimer’s and “lost time.”
6. In early 2025 the Respondent was depicted in a video screaming and yelling at another person, trying to hit them, drooling, and spitting at the person. The Respondent claimed that the video was the result of “reactive abuse” and “psychosis” and was not normal for her.
7. The Respondent further explained that, following the dog-bite, the Respondent’s physical condition and health deteriorated. Despite seeking medical advice, she suffered pain and cognitive problems. According to the Respondent her condition was not correctly diagnosed until February of 2026 in which she was diagnosed with MRSA and a sepsis infection which caused swelling of her brain and almost causing her to die. The Respondent was hospitalized on February 17, 2026, for several weeks. She returned to seeing clients in March of 2026.
8. Three complaints were filed in February and March of 2026 with OPR regarding the practice of the Respondent. These caused an investigation to begin. Investigator Kris Bowdish investigated the complaints and interviewed the Respondent twice.
9. The Respondent admitted to Investigator Bowdish that she had suffered memory loss, seizures, delirium, psychosis, and a feeling of Alzheimer’s on and off from 2024 to the present.
10. The Respondent admitted that when she was having symptoms of memory loss, she had no insight or knowledge that her cognition was impaired.

11. For the majority of this period, she was treating clients.
12. The Respondent testified that in her opinion, the Respondent is safe to practice. At a later point she stated that she was not fully recovered yet and that she intended to “refinance” so she could take a break of four to six weeks to “feel normal” even though she resumed seeing clients in March of 2026.
13. The Respondent also testified that she was competent to practice because if she makes an error, she can make “repairs.”
14. The Respondent appeared to believe that, because her mental and behavioral problems were caused by a medical condition, that she was not responsible for the lapses in professional behavior. She had little insight into the effect of her condition regarding her treatment of clients and compliance with professional responsibilities.

Dual Relationships

15. In October of 2025, the Respondent began treating a client by the initials “E.B.”
16. At some point, “E.B.” became friends with the Respondent’s daughter. On December 30, 2025, the Respondent ended her therapeutic relationship with “E.B.” The Respondent offered “E.B.” that the Respondent would personally pay for the expense of an alternate therapist. About two weeks after the last appointment, in January of 2026, “E.B.” and her three-year old child came to live in the home of the Respondent and her daughter. According to the Respondent, this was hard to shift from a therapist/client relationship to a different relationship. She described the new relationship as “a simulated parental relationship.” The Respondent acknowledged that the shift from therapist to “simulated parent relationship” was inappropriate but she felt that she had no choice because of the housing problem of “E.B.” and her child.
17. In February of 2026 the Respondent, the Respondent’s daughter, and “E.B.” went on a trip to Lake George, NY, for a two-night recreation trip. “E.B.” moved out from the Respondent’s home shortly after the trip to Lake George and has not returned.
18. The Respondent told Investigator Bowdish that she thought the period of time before a therapist could have a relationship with a former client was seven years.
19. The Respondent admitted that allowing “E.B.” to live in her home was a “very grave error” for which she took responsibility because she had no other choice but to offer the housing to “E.B.”
20. The Respondent has a supervising clinician, Kathleen Paine. The Respondent did not consult with her supervising clinician before deciding to allow “E.B.” to move into her home. Ms. Paine was not offered as a witness to the Respondent’s current capacity to practice. No other witness with professional qualifications was offered to support the Respondent’s claim that she is capable to practice safely.

Records and Billing

21. The Respondent also told Investigator Bowdish that, at one point, the Respondent had confused client “E.B.” with another client who had the same first name. The billing and chart notes had become confused.

22. The Respondent told Investigator Bowdish that the Respondent was behind on about 58 clinical notes for clients.

Conclusions of Law

1. The Hearing Authority concludes that the Respondent had violated the following statutes regarding unprofessional practice: 3 VSA Sec. 129(a)(5) [Practicing the profession when medically or psychologically unfit to do so]; 3 VSA Sec. 129a(a)(28) [Engaging in conduct of a character likely to deceive, defraud, or harm the public]; 3 VSA Sec. 129a (a)(15) [Failing to exercise independent professional judgment in the performance of licensed activities when that judgment is necessary to avoid action repugnant to the obligations of the profession]; and 3 VSA Sec. 129a(b)(2) [Failure to practice competently by reason of any cause on a single occasion or on multiple occasions whether actual injury to a client, patient or customer has occurred. Failure to practice competently includes: failure to conform to the essential standards of acceptable and prevailing practice.] The National Board of Certified Counselors Code of Ethics, revised 8/24/2023, urges practitioners to avoid multiple relationships with clients. See paragraph 22 of Code of Ethics.
2. In particular, the Board is concerned that the Respondent does not have, and did not have, insight into the level of her incapacity to safely practice while her medical condition flared. The facts found above were proven by a preponderance of the evidence.
3. Those facts establish that, in order to protect the public health, safety and/or welfare of the people of the State of Vermont, emergency action is imperative and the license of the Respondent should be summarily suspended. The Findings of Fact and Conclusions of Law in this Summary Suspension hearing are for purposes of deciding whether **at this time** there is an imperative need to take emergency action. 3 V.S.A. § 814(c). The Findings of Fact and Conclusions of Law herein are for purposes of this Order only.
4. Vermont law requires that unprofessional charges be filed promptly with Respondent being afforded a prompt hearing. At any merits hearing in this matter, the prosecutor will bear the burden of proving unprofessional conduct. The Findings and Conclusions in this matter will not absolve the prosecution or Respondent from producing or challenging relevant evidence at a merits hearing. The Findings of Fact and Conclusions of Law at a contested hearing will be based exclusively on the evidence admitted at that hearing.

Order

The Request for Summary Suspension is **Granted**. Respondent's LCMHC license is **Summarily Suspended** and shall not be subject renewal or reinstatement during the period of Summary Suspension without Board approval pending proceedings. These proceedings shall be promptly instituted and determined. 3 V.S.A. § 814(c).

SO ORDERED.

Vermont Board of Allied Mental Health Practitioners

By: DocuSigned by: *Tammy Austin* Date 4/20/2026
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OFFICE OF PROFESSIONAL REGULATION

DATE OF ENTRY: 4/21/2026

Appeal Rights

This is a final administrative determination by the Vermont Board of Allied Mental Health Practitioners. A party aggrieved by a final decision of a board or hearing authority may appeal this decision by filing a written Notice of Appeal with the Director of the Office of Professional Regulation, Vermont Secretary of State, 89 Main Street, Fl. 3, Montpelier, VT 05620-3402 within 30 days of the entry of this order. If an appeal is filed, the Director of the Office of Professional Regulation shall assign the case to an appellate officer. The review shall be conducted on the basis of the record created before the board. In cases of alleged irregularities in procedure before the board, not shown in the record, proof on that issue may be taken by the appellate officer. 3 V.S.A. §§ 129(d) and 130a.